

OREGON STATE HOSPITAL

POLICY ATTACHMENT

PROCEDURES A: Discharge Readiness

POLICY: 6.013

POINT PERSON: Director of Social Work

APPROVED: Chief Medical Officer

DATE: SEPTEMBER 2, 2026

SELECT ONE:

☐ New policy attachment

☒ Revision of existing policy attachment

☐ Reaffirmation of existing
policy attachment

RESPONSIBLE PERSON/GROUP	PROCEDURES
Interdisciplinary Treatment Team (IDT)	1. Include the patient as much as possible when making plans for discharge or conditional release. 2. Find issues that could delay discharge and: a. Create Treatment Care Plan (TCP) goals to address each issue. b. Set target dates for Completion. c. Assign team members to help remove these barriers. 3. Decide if the patient is: a. Ready to transition, or b. Ready for conditional release. (This decision may be made by the IDT, Risk Review Panel, or Program Executive Team, depending on jurisdiction type.) 4. Record the patient's discharge readiness in the medical record following department rules. 5. If the patient is under the PSRB and can be safely supervised in the community, request a Risk Review Panel decision on conditional release readiness. 6. Give the patient's guardian a chance to take part in discharge planning. 7. If a patient refuses to take part in discharge planning: a. Document the refusal, b. Explain why the patient refused, c. List of what plans were offered, and d. Describe efforts made to help the patient. 8. Use clinical judgment and ask the patient who they want notified about their discharge. a. Identify who should be legally notified.

	b. Notify the required people. 9. Check the patient's suicide risk following OSH Policy 6.056, Suicide Risk Screening and Assessment. 10. Include input from the patient's caregiver and other supports when appropriate. 11. If a patient is discharged unexpectedly (for example, due to court action or a voluntary discharge request): a. Try to connect the patient with community services. b. Document all efforts made.
Psychiatrist/PMHNP	1. Complete required discharge readiness assessments: a. For ORS 161.370 patients: complete a Hospital Level of Care Assessment at the 10-Day Review and each TCP meeting. b. For ORS 426.130 and 426.220 patients: complete a Ready to Transition Assessment at every TCP meeting with Social Work and the IDT. 2. Write a progress note stating: a. Whether the patient is ready for discharge, and b. Any barriers and steps to address them if the patient is not ready. 3. Work with the social worker and patient on discharge planning. 4. Complete and sign the Discharge Medication Reconciliation Orders, which also serve as the discharge orders, and include the discharge location. 5. Complete prescriptions for medications appropriate for the patient and their discharge location, including any necessary medications for opioid overdose reversal, in alignment with OAR 333-505-0055. 6. Complete the Psychiatry Discharge Summary within 20 days of discharge, including: a. Hospital course, b. Continuing care plan, and c. Treatment recommendations.
Pharmacy	1. Process discharge medication orders and supply to unit, including any required medications for opioid overdose reversal.
Registered Nurse (RN)	2. Follow Nursing Protocol 2.027, Discharge of Patients, and complete the Nursing Discharge Bundle within 24 hours before discharge. This includes: a. Nursing Discharge Assessment, and b. Suicide Risk Screening at discharge. 3. Review discharge instructions with the patient and caregiver when possible.

	4. Prepare medications, supplies, and medical equipment to go with the patient as required.
Social Worker	1. Document discharge planning and community integration progress throughout the hospital stay. 2. Complete a Continuing Care Discharge Plan (CCDP) that includes: a. Housing and supervision, b. Addresses and contact persons, c. Support systems, d. Medical and mental health needs, e. Appointments, f. Special care needs, g. Likelihood of needing post-hospital services, and h. Available services and recommendations, per ORS 441.196. 3. Schedule follow-up appointments within 7 days of discharge. 4. If follow-up appointments cannot be scheduled within 7 days, document the reason. 5. For patients discharged to another institution (such as jail or a medical hospital): a. Include mental health care recommendations for that setting and for future community discharge. 6. If the patient or guardian refuses to sign the CCDP: a. Write "patient (or guardian) refuses to sign," and b. Initial the statement. 7. Attach resource lists based on the patient's needs, such as (when applicable): a. Skilled nursing facilities, b. Intermediate care facilities, c. Substance use disorder treatment services, d. Housing options, e. Shelters, f. Food banks. 8. Arrange transportation for patients who are civilly committed, voluntary by guardian, at the end of their jurisdiction under the PSRB, or conditionally released under the PSRB. 9. Notify outside providers and partners of the discharge. a. Make phone contact if the IDT identifies immediate risks. b. Document all contact. 10. Sign the CCDP and send copies to: a. The patient, b. Care providers,

	c. Community mental health representatives, and d. Other involved agencies at discharge.
Health Information	1. After the Psychiatry Discharge Summary is completed: a. Send the CCDP and other required clinical documents to continuing care providers. 2. Document the date all information was shared.